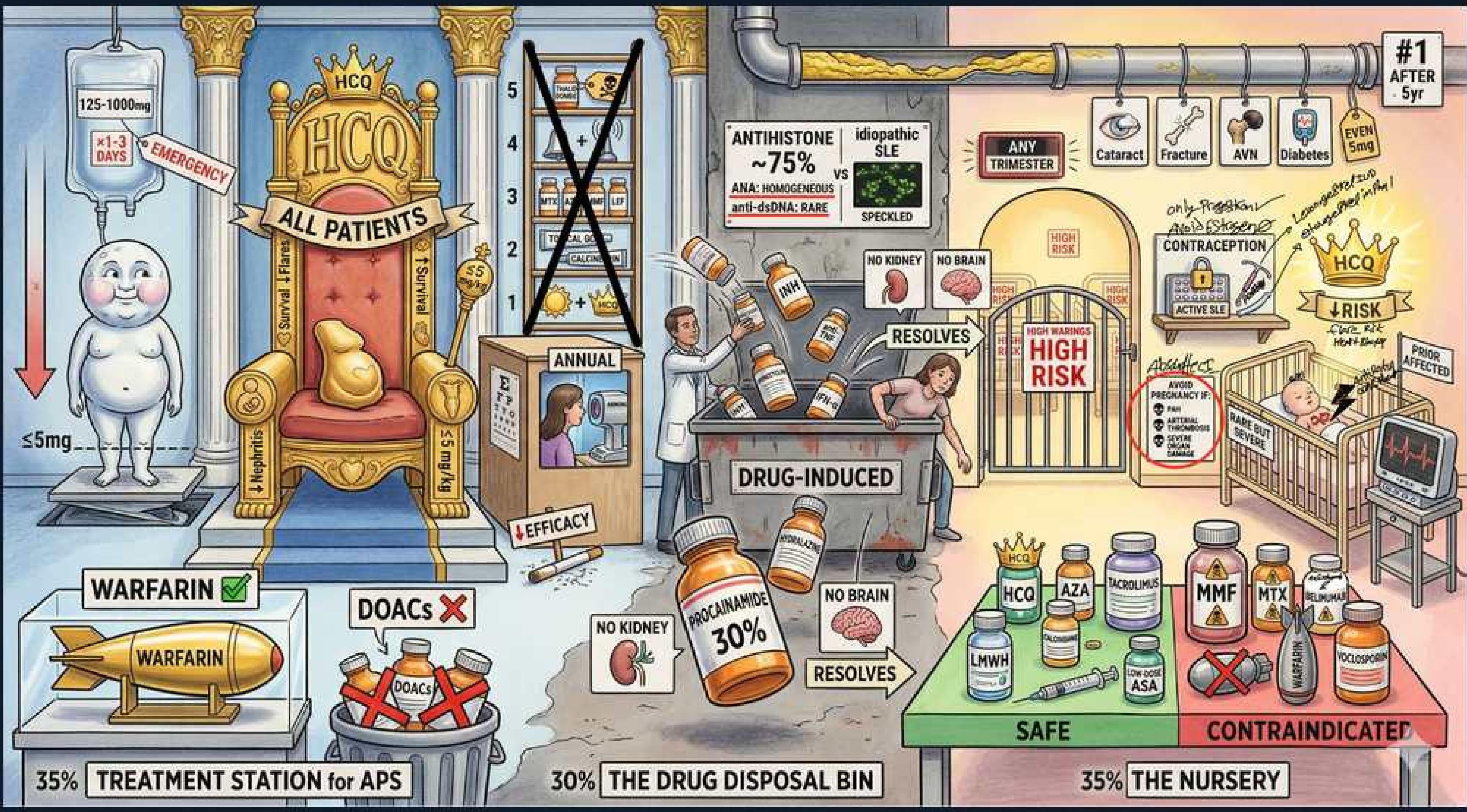
SLE — APS, Pregnancy, Neonatal & Drug-Induced



1. HCQ for ALL patients

WHAT YOU SEE

'HCQ — ALL PATIENTS' throne

WHAT IT ENCODES

Hydroxychloroquine (typically 200–400 mg/day, dosed ≤5 mg/kg actual body weight to limit retinopathy) is the backbone for essentially ALL SLE patients because it reduces flares, organ damage, thrombosis, and mortality, and improves lipids and pregnancy outcomes. It is CONTINUED throughout pregnancy and lactation. Baseline and periodic (annual after 5 years) ophthalmologic screening detects bull's-eye maculopathy.

BOARD TRAP

Stopping HCQ in pregnancy is the classic trap and the wrong answer — discontinuation roughly doubles flare risk and increases congenital heart block in anti-Ro-positive mothers, so it must be continued. The only routine monitoring 'gotcha' is retinopathy screening, not pregnancy discontinuation.

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2. APS treatment — warfarin

WHAT YOU SEE

'WARFARIN □' missile

WHAT IT ENCODES

Antiphospholipid syndrome with a confirmed thrombotic event requires long-term (often lifelong) anticoagulation with a vitamin-K antagonist (warfarin), target INR 2–3 for venous events. APS is diagnosed by a clinical event (thrombosis or pregnancy morbidity) PLUS a persistently positive antibody (lupus anticoagulant, anti-cardiolipin, or anti-β2-glycoprotein I) on two occasions ≥12 weeks apart. Lupus anticoagulant paradoxically prolongs the aPTT in vitro but causes clotting in vivo.

BOARD TRAP

Do not anticoagulate for a positive antiphospholipid antibody ALONE without a clinical event — asymptomatic carriers get low-dose aspirin and risk-factor control, not warfarin. The prolonged aPTT that fails to correct with mixing study is the lupus anticoagulant (a clotting risk), not a bleeding risk — treating it as a factor deficiency is the trap.

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3. DOACs contraindicated in APS

WHAT YOU SEE

'DOACs □'

WHAT IT ENCODES

DOACs (especially rivaroxaban) are inferior to warfarin in APS and are CONTRAINDICATED in high-risk, triple-positive patients (positive for lupus anticoagulant + anti-cardiolipin + anti-β2GPI) and arterial thrombosis, due to higher recurrent-thrombosis rates in trials. Use warfarin (VKA) with INR 2–3 instead.

BOARD TRAP

Choosing a DOAC for APS thrombosis is the trap — warfarin is the standard, and DOACs are specifically avoided in triple-positive/arterial APS. In pregnancy, warfarin is also wrong (teratogenic) and the answer becomes LMWH + aspirin.

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4. APS in pregnancy: LMWH + ASA

WHAT YOU SEE

Heparin drip '125-1000mg', baby

WHAT IT ENCODES

Obstetric APS is treated with low-dose aspirin (started pre-/early pregnancy) PLUS prophylactic or therapeutic low-molecular-weight heparin, because heparin does not cross the placenta. This regimen reduces recurrent miscarriage and late-pregnancy thrombosis/morbidity. Warfarin is switched to LMWH before/at conception because it crosses the placenta.

BOARD TRAP

Continuing warfarin into pregnancy is the trap: warfarin is teratogenic (fetal warfarin syndrome — nasal hypoplasia, stippled epiphyses, CNS anomalies), so switch to LMWH. Conversely, using LMWH + aspirin is correct for pregnancy but warfarin remains the answer for non-pregnant long-term APS — match the regimen to the pregnancy status.

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5. Drug-induced lupus

WHAT YOU SEE

'DRUG-INDUCED' tipped pill barrel

WHAT IT ENCODES

Drug-induced lupus erythematosus (DILE) presents with arthralgias/arthritis, myalgias, fever, and serositis (pleuritis/pericarditis) developing after months of exposure to a culprit drug, with an even sex ratio and older patients than idiopathic SLE. It characteristically SPARES the kidney and CNS. Anti-histone antibodies are positive in ~95% of classic DILE (procainamide/hydralazine).

BOARD TRAP

DILE is the answer when an older patient on a chronic drug develops arthralgias and serositis WITHOUT nephritis or CNS disease — picking idiopathic SLE misses the spared organs, the even sex ratio, and the anti-histone/normal-complement profile. Note anti-TNF-induced lupus is an exception that can be anti-dsDNA positive.

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6. Anti-histone ~75%

WHAT YOU SEE

'ANTIHISTONE ~75%', homogeneous ANA, anti-dsDNA rare

WHAT IT ENCODES

DILE classically shows a positive ANA (homogeneous pattern) and anti-HISTONE antibodies in the large majority (~75–95%), with anti-dsDNA and anti-Smith typically ABSENT and complement usually NORMAL. Anti-histone is sensitive for DILE but NOT specific — it also appears in ~50% of idiopathic SLE.

BOARD TRAP

Anti-histone is sensitive but NOT specific to DILE, so its presence alone doesn't prove DILE — the discriminator is the ABSENCE of anti-dsDNA, normal complement, and spared kidney/CNS. Picking anti-dsDNA or low complement as the expected DILE finding is the trap (those indicate idiopathic SLE).

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7. Procainamide / hydralazine

WHAT YOU SEE

'PROCAINAMIDE 30%' pill

WHAT IT ENCODES

Highest-risk culprits are PROCAINAMIDE (greatest risk, ~20–30%) and HYDRALAZINE (next), followed by isoniazid, minocycline, quinidine, methyldopa, chlorpromazine, and anti-TNF agents. Slow acetylators are predisposed (hydralazine, procainamide, isoniazid are acetylated). Mnemonic for the top offenders: 'HIPP' — Hydralazine, Isoniazid, Procainamide, Phenytoin.

BOARD TRAP

Procainamide carries the HIGHEST risk of inducing lupus, even though hydralazine is encountered more often clinically — confusing which drug is highest-risk is a common trap. Also, the SCLE-inducing drugs (hydrochlorothiazide, terbinafine, CCBs, PPIs) differ from the systemic-DILE list, so match the drug to the pericardial phenotype.

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8. DILE spares kidney & CNS

WHAT YOU SEE

'NO KIDNEY' and 'NO BRAIN' signs

WHAT IT ENCODES

A defining feature of drug-induced lupus is that it characteristically SPARES the kidney (no significant nephritis) and the CNS, and complement stays normal. This is the single most useful discriminator from idiopathic SLE on a board stem. Management is simply STOPPING the offending drug (NSAIDs or short steroids for symptoms).

BOARD TRAP

If the stem includes glomerulonephritis, active urine sediment, low complement, or neuropsychiatric disease, it is idiopathic SLE, NOT DILE — choosing DILE in the presence of renal/CNS involvement is the trap. The presence of these organs being involved overrides a positive anti-histone.

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9. DILE resolves on withdrawal

WHAT YOU SEE

A RESOLVES tag near the tipped drug barrel — symptoms clear once the culprit drug is stopped; the fix is withdrawal, not immunosuppression.

WHAT IT ENCODES

Symptoms of DILE resolve over days to weeks (sometimes months) after STOPPING the culprit drug; serologies (anti-histone, ANA) can take longer to normalize. The first-line management is drug discontinuation, with NSAIDs or a short steroid course for persistent serositis/arthritis. Long-term immunosuppression is usually unnecessary.

BOARD TRAP

The correct first step is to STOP the drug, not start chronic immunosuppression — escalating to cyclophosphamide or long-term steroids for DILE is the over-treatment trap. Failure to resolve after withdrawal should make you reconsider idiopathic SLE.

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10. Anti-Ro/La → neonatal lupus

WHAT YOU SEE

A nursery crib with a cardiac monitor (THE NURSERY) — maternal anti-Ro/La crossing the placenta: transient rash but PERMANENT congenital heart block.

WHAT IT ENCODES

Maternal anti-Ro/SSA (and anti-La/SSB) IgG cross the placenta and cause neonatal lupus: a transient annular/photosensitive rash, cytopenias, and transaminitis that resolve as maternal antibody clears (~6 months), PLUS congenital complete heart block, which is PERMANENT. Mothers may be asymptomatic or have SLE/Sjögren. Anti-Ro-positive pregnancies get serial fetal echocardiograms (~weeks 16–26) to detect emerging block; HCQ reduces the risk.

BOARD TRAP

The cutaneous and hematologic features of neonatal lupus are TRANSIENT, but the congenital complete heart block is PERMANENT and often requires a pacemaker — assuming the heart block will resolve like the rash is the trap. The responsible antibody is anti-Ro/SSA (not anti-dsDNA), and a negative maternal ANA does not exclude it.

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11. High-risk pregnancy gate

WHAT YOU SEE

'HIGH RISK' red gate

WHAT IT ENCODES

SLE pregnancy should be planned during stable remission for ≥6 months on pregnancy-compatible medications, because active disease (especially lupus nephritis) at conception predicts flares, preeclampsia, preterm birth, IUGR, and fetal loss. Baseline labs (anti-Ro/La, antiphospholipid antibodies, complement, anti-dsDNA, urine protein) risk-stratify. Distinguishing a lupus nephritis flare from preeclampsia is a key late-pregnancy challenge.

BOARD TRAP

Preeclampsia vs lupus nephritis flare: BOTH cause hypertension and proteinuria after 20 weeks, but a lupus FLARE shows rising anti-dsDNA, FALLING complement, active urinary sediment (RBC casts), and other-organ activity, whereas preeclampsia has normal/rising complement, high uric acid, and elevated sFlt-1/PlGF ratio — calling a flare 'just preeclampsia' (or vice versa) is the trap, since management diverges (immunosuppression vs delivery).

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12. Safe in pregnancy: HCQ/AZA

WHAT YOU SEE

'SAFE' green shelf: HCQ, AZA, LMWH, ASA

WHAT IT ENCODES

Pregnancy-compatible SLE drugs: HYDROXYCHLOROQUINE (continue in all), AZATHIOPRINE (steroid-sparing, ≤2 mg/kg), low-dose glucocorticoids (prednisone, which is largely placentally metabolized), LMWH, low-dose aspirin, and calcineurin inhibitors (tacrolimus). For severe flares, IV methylprednisolone and IVIG are options. Aspirin is also given to most lupus pregnancies for preeclampsia prophylaxis.

BOARD TRAP

Switching a stable pregnant patient OFF hydroxychloroquine or azathioprine 'to protect the fetus' is the trap — both are safe and stopping them risks a flare. Note the fetus cannot metabolize fluorinated steroids (dexamethasone/betamethasone), so those are reserved for fetal indications, not maternal maintenance.

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13. Contraindicated: MMF/MTX/CYC

WHAT YOU SEE

'CONTRAINDICATED' red shelf: MMF, MTX

WHAT IT ENCODES

Teratogens that must be STOPPED before conception: MYCOPHENOLATE mofetil (cleft lip/palate, ear/limb anomalies — switch to azathioprine ~3 months pre-pregnancy), METHOTREXATE (abortifacient/teratogen — stop ≥1–3 months prior, supplement folate), and CYCLOPHOSPHAMIDE (teratogenic and gonadotoxic; avoid except life-threatening flares in 2nd/3rd trimester). WARFARIN is also teratogenic (covered under APS).

BOARD TRAP

The classic swap is MYCOPHENOLATE → AZATHIOPRINE before pregnancy — continuing MMF or methotrexate into pregnancy is the major teratogen trap. Don't confuse: azathioprine is SAFE while its 'cousin' mycophenolate is contraindicated, and methotrexate must be stopped with folate supplementation.

14. Steroid risks (long-term)

WHAT YOU SEE

Cataract, fracture, AVN, diabetes icons

WHAT IT ENCODES

Chronic glucocorticoids cause cumulative harm: posterior subcapsular cataracts and glaucoma, osteoporosis with fragility fractures, AVASCULAR NECROSIS (classically femoral head — disproportionate hip/groin pain), steroid-induced diabetes/hyperglycemia, hypertension, weight gain/Cushingoid features, infection, and adrenal suppression. Minimize dose and duration; add calcium/vitamin D and bisphosphonate prophylaxis for prolonged courses, and use steroid-sparing agents.

BOARD TRAP

In a lupus patient on chronic steroids with new groin/hip pain and NORMAL early plain films, the answer is avascular (osteonecrosis) of the femoral head diagnosed by MRI — attributing it to a lupus arthritis flare or waiting for X-ray changes is the trap. Steroids (not the lupus itself) are the leading cause of AVN here.